SAFETY & MONITORING

Safety and monitoring

Contraindications, warnings, monitoring and adverse reactions. This summary does not replace the full prescribing information.[1]

Contraindications | Warnings | Monitoring | Adverse reactions

Contraindications

Other conditions that need caution are covered under warnings and precautions.

Contraindicated

Oral olanzapine is contraindicated in patients with:[1]

- Hypersensitivity to olanzapine or to any of the excipients
- Known risk of narrow-angle glaucoma

Excipients include lactose monohydrate. See the prescribing information for the full list.

Important warnings and precautions

During antipsychotic treatment, improvement in the patient's clinical condition may take several days to some weeks. Patients should be closely monitored during this period.[1]

Dementia-related psychosis and/or behavioural disturbances

Olanzapine is not recommended for patients with dementia-related psychosis and/or behavioural disturbances because of an increase in mortality and the risk of cerebrovascular accident. In placebo-controlled trials (6–12 weeks) in elderly patients (mean age 78 years):[1]

3.5% vs 1.5%

incidence of death, olanzapine vs placebo (2-fold increase)

1.3% vs 0.4%

cerebrovascular adverse events, olanzapine vs placebo (3-fold increase)

The efficacy of olanzapine was not established in these trials.

Key warnings

Metabolic effects

Olanzapine treatment has been associated with weight gain and changes in glucose and lipid parameters. Hyperglycaemia and the development or worsening of diabetes, occasionally associated with ketoacidosis or coma, have been reported uncommonly, including some fatal cases.[1]

Assess relevant baseline risk factors and monitor weight, blood glucose and lipid parameters as set out in the monitoring schedule. See monitoring.

Somnolence and dizziness

Somnolence and dizziness may occur. Patients should be cautioned about operating machinery, including motor vehicles.[1]

Sedation and orthostatic hypotension should be considered when assessing falls risk, concomitant medication and individual tolerability.

Other warnings

Parkinson's disease

Not recommended for dopamine agonist-associated psychosis in Parkinson's disease; worsening of parkinsonian symptoms and hallucinations was reported very commonly.[1]

QT interval

Caution with medicines known to increase QTc, especially in the elderly and in congenital long QT syndrome, congestive heart failure, heart hypertrophy, hypokalaemia or hypomagnesaemia.[1]

Thromboembolism

Venous thromboembolism has been reported uncommonly. Identify risk factors such as immobilisation and take preventive measures.[1]

Seizures

Use cautiously in patients with a history of seizures or factors that may lower the seizure threshold.[1]

Neuroleptic malignant syndrome

Neuroleptic malignant syndrome is a potentially life-threatening condition associated with antipsychotic medicines. Clinical manifestations include hyperpyrexia, muscle rigidity, altered mental status and autonomic instability.[1]

If a patient develops signs and symptoms indicative of NMS, or presents with unexplained high fever without additional clinical manifestations of NMS, **all antipsychotic medicines, including olanzapine, must be discontinued**.

Tardive dyskinesia and other movement disorders

Movement-related adverse effects may occur. The risk of tardive dyskinesia increases with long-term exposure; patients should be assessed for relevant symptoms during treatment.[1]

If signs or symptoms of tardive dyskinesia appear, dose reduction or discontinuation should be considered. These symptoms can temporarily deteriorate or even arise after discontinuation of treatment.

Cardiovascular and orthostatic effects

Orthostatic hypotension and dizziness may occur. Caution is required in patients with relevant cardiovascular risk factors or when medicines with potential interactions are co-administered. Blood pressure should be measured periodically in patients over 65 years.[1]

Sudden cardiac death

Reported post-marketing. In an observational study, the risk with olanzapine was approximately twice that in patients not using antipsychotics, and comparable to pooled atypical antipsychotics.[1]

Anticholinergic activity

Caution in prostatic hypertrophy, paralytic ileus and related conditions.[1]

General CNS activity

Caution with alcohol and other centrally acting medicines. Olanzapine may antagonise the effects of dopamine agonists.[1]

Lactose

Patients with rare hereditary galactose intolerance, total lactase deficiency or glucose-galactose malabsorption should not take this medicine.[1]

Hepatic and haematological considerations

Transient, asymptomatic elevations of hepatic aminotransferases (ALT, AST) have been seen commonly, especially in early treatment. Follow-up is important in patients with elevated liver enzymes, signs of hepatic impairment, limited hepatic functional reserve or concomitant use of potentially hepatotoxic medicines. If hepatitis is diagnosed, olanzapine treatment should be discontinued.[1]

Caution is also required in patients with low leukocyte or neutrophil counts, medicines known to cause neutropenia, a history of drug-induced bone marrow depression, or hypereosinophilic or myeloproliferative conditions. Neutropenia has been reported commonly when olanzapine and valproate are used concomitantly.

Monitoring during treatment

Example intervals in line with antipsychotic guidelines. Individualise monitoring to the patient's clinical circumstances.[1]

Table 1. Example monitoring schedule

Parameter	Baseline	Week 4	Week 8	Week 12	Thereafter
Body weight	Yes	Yes	Yes	Yes	Quarterly
Blood glucose	Yes			Yes	Annually
Lipids	Yes			Yes	Every 5 years
Blood pressure	Periodically in patients over 65 years				

Patients should be observed for signs and symptoms of hyperglycaemia (such as polydipsia, polyuria, polyphagia and weakness). Patients with diabetes mellitus or with risk factors for diabetes should be monitored regularly for worsening of glucose control.[1]

See the full prescribing information for complete monitoring recommendations.

Weight and metabolic changes over time

The proportion of patients with clinically significant changes in weight, glucose and lipids increased with longer exposure (at least 48 weeks).[1]

64.4%

of adults gained ≥ 7% of baseline weight with long-term exposure (≥ 48 weeks)[1]

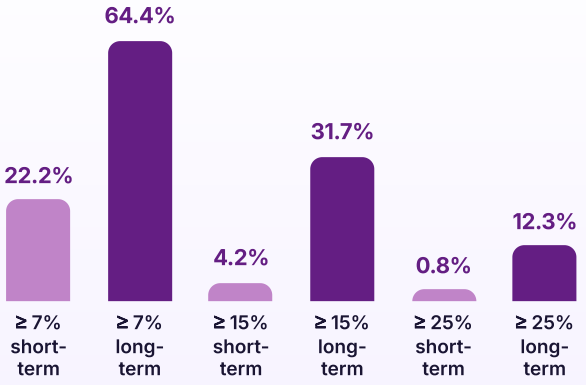
22.2%

gained ≥ 7% with short-term treatment (median 47 days)[1]

39.9%

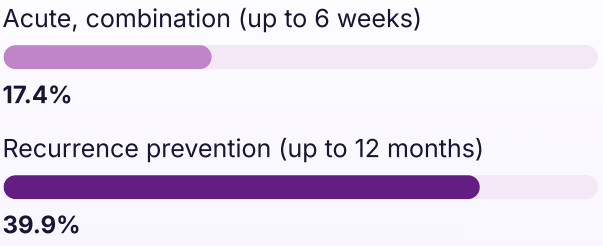
gained ≥ 7% with up to 12 months of recurrence-prevention treatment[1]

All adults, by exposure



Percentage of patients reaching each weight-gain threshold. Clinically significant weight gain was observed across all baseline BMI categories.

Bipolar disorder clinical studies



Patients gaining ≥ 7% of baseline body weight.

Weight-gain thresholds across short-term and long-term exposures and bipolar disorder clinical trials.[1]

Drug interactions

Olanzapine metabolism may be affected by medicines or substances that influence CYP1A2 activity. Review concomitant medicines, smoking status and relevant changes during treatment.[1]

Table 2. Selected interactions

Medicine or factor	Effect	Recommended action
Smoking; carbamazepine (CYP1A2 induction)	May increase olanzapine metabolism and reduce concentrations; only slight to moderate increase in clearance observed	Clinical monitoring; a dose increase may be considered if necessary
Fluvoxamine; other CYP1A2 inhibitors such as ciprofloxacin	Increase olanzapine exposure. With fluvoxamine, mean Cmax rose 54% (female non-smokers) and 77% (male smokers); mean AUC rose 52% and 108%	Consider a lower starting dose; consider a dose decrease if an inhibitor is started
Activated charcoal	Reduces oral bioavailability by 50–60%	Take at least 2 hours before or after olanzapine
Alcohol and other centrally acting medicines	Additive CNS effects	Caution
Direct and indirect dopamine agonists	Olanzapine may antagonise their effects	Anti-Parkinsonian medicines not recommended in patients with Parkinson's disease and dementia
Medicines that increase QTc	Potential additive effect	Caution
Valproate	Neutropenia reported commonly with concomitant use	See hepatic and haematological considerations

Olanzapine does not inhibit the main CYP450 isoenzymes in vitro, and showed no interaction with lithium or biperiden.[1]

Pregnancy and breastfeeding

Because human experience is limited, olanzapine should be used in pregnancy only if the potential benefit justifies the potential risk to the foetus.[1]

Newborns exposed to antipsychotics, including olanzapine, during the third trimester are at risk of adverse reactions including extrapyramidal and/or withdrawal symptoms, and should be monitored carefully.

Olanzapine is excreted in breast milk. Patients should be advised not to breastfeed an infant if they are taking olanzapine.

Adverse reactions

Frequently reported adverse reactions include somnolence, weight gain, increased appetite, dizziness, orthostatic hypotension and changes in glucose and lipid parameters. The full prescribing information gives the complete profile and frequency categories.[1]

Full adverse-reaction table →

~30%

prolactin above normal in trials up to 12 weeks (normal baseline); generally mild[1]

Very common

somnolence and weight gain (≥ 1 in 10 patients)[1]

ALL ADVERSE REACTIONS REPORTED IN ≥ 1% OF ADULTS IN CLINICAL TRIALS

26 reported · Clinical trials

- Somnolence
- Weight gain
- Eosinophilia
- Elevated prolactin
- Elevated cholesterol, glucose and triglyceride levels
- Glucosuria
- Increased appetite
- Dizziness
- Akathisia
- Parkinsonism
- Leukopenia
- Neutropenia
- Dyskinesia
- Orthostatic hypotension
- Anticholinergic effects
- Transient asymptomatic elevations of hepatic aminotransferases
- Rash
- Asthenia
- Fatigue
- Pyrexia
- Arthralgia
- Increased alkaline phosphatase
- High gamma glutamyltransferase
- High uric acid
- High creatine phosphokinase
- Oedema

Frequencies as listed for adults in clinical trials.[1]

PHARMACOVIGILANCE

Reporting of suspected adverse reactions

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product. Healthcare professionals are asked to report any suspected adverse reactions via their national reporting system.[1]

PRODUCT INFORMATION

Full prescribing information

The full prescribing information and the patient leaflet for ZYPREXA coated tablets and ZYPREXA powder for solution for injection.
Both links open the European Medicines Agency website.

View full prescribing information

Access patient leaflet

← Previous screen: **Dosing**

Next screen: **Clinical evidence** →

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